



Granting and Management of Leave for Mental Health Inpatients Policy

1. Applicability

Site	Operational Area	Applicable to
Armadale Mental Health Service	All areas	All workforce groups

2. Purpose

Leave for admitted mental health inpatients is encouraged to support person-centred recovery, sustain community-based living and ensure that treatment is provided in the least restrictive manner possible. Leave may also be a key component of discharge planning.

3. Scope

This document outlines the requirements to be met when granting leave to mental health inpatients. Decisions to support, or decline, leave requests are based on the assessment and management of clinical risk and should be made through active collaboration with the treating team, the patient, carers and families. All decisions regarding a patient's leave and the rationale behind decision making is to be documented in full in the patient's Medical Record.

4. Risk Assessment

Regardless of the legal status of the patient under the [Mental Health Act 2014](#), the allocated clinician and/or treating medical officer are to review the patient's current risk assessment and mental state before each episode of leave, in collaboration with the patient, carer/s and/or escort where appropriate.

Risk Assessment

This assessment, together with any identified risk mitigation strategies must be documented in the patient's medical record at the earliest opportunity prior to leave commencing. Consideration is to be given to the patient's:

- Risk to self;
- Risk to others;
- Risk/vulnerability from others;
- Physical health status.
- Risk of AWOL
- Assessment of current mental state.

Risks will be weighed against the positive benefits of empowering the patient to take more responsibility and control of their life and recognition of the cultural, diverse, and individual needs of the patient, carer and family. Particular consideration may be needed for people of Aboriginal or Torres Strait Islanders descent who may require cultural leave.

When assessing the granting of leave, a decision is to be made whether the leave granted is to be escorted or unescorted leave. When escorted leave is granted, the multidisciplinary team will consider the role of the escort relative to the patient when determining an escort that is not a clinician. Information will be gathered from stakeholders involved with the patient and family.

It is recommended that newly admitted involuntary consumers not be granted leave, even for brief periods, until the treating team has developed enough familiarity with them to make a valid assessment of their mental state and risk.

Risk mitigation strategies will be documented and shared with the patient and family / carer prior to the commencement of leave.

Refer to the [Mental Health Act 2014](#), s. 105, 108-110.

If a patient has impaired decision-making capacity, a substitute decision maker is to be identified for the purpose of leave management and this information recorded in the patient's medical record.

5. Granting leave

Refer to the [Mental Health Act 2014](#) s. 105, 108-110.

Refer to the following policies/procedures:

- Granting of leave procedure within this policy.
- [AKG Consumer Safety Planning Procedure](#)

Smoking

Granting leave from the ward or unit specifically to smoke is not in accordance with the [Smoke Free Policy](#) as it negates the physical and mental health benefits of going smoke free and as such specific 'smokers leave' has no place in a smoke free health service. Seek advice from senior staff as relevant to support care planning and review the [AKG Tobacco \(Nicotine\) – Patient management Guideline](#).

6. Voluntary patients

Although voluntary patients are free to leave the hospital premises, staff are to communicate the local processes and general responsibilities held by both voluntary patients and staff for supporting treatment adherence and safety during periods of leave. Patients, carers and families are encouraged to inform staff when they are taking periods of leave. Staff are to ensure patients, carers and families are provided the opportunity to discuss leave arrangements prior to leave, on all occasions of leave.

Leave for voluntary patients under the age of 18 years who are admitted to the adult IPU should adhere to the same principles of risk assessment, safety planning, balanced with promotion of autonomy and a recovery orientation. Particular attention to the involvement of family and carers in this assessment and their support for such leave is vital.

Absence from the ward is to be documented in the patient's medical record and communicated to the Clerk, Shift Coordinator and the Care Coordinator to facilitate staff awareness of leave and management of any absence.

Voluntary patients who unexpectedly leave the IPU without notifying staff are to have a risk assessment performed (including attempts to contact the patient to verify safety) based on the

most recent clinical contact with the patient. If the voluntary patient is assessed as high risk, action is to be taken in line with the [EMHS Missing or Suspected Missing Inpatient Procedure](#).

7. Involuntary patients

Involuntary patients are only to be granted leave of absence by a psychiatrist. The psychiatrist's decision to grant leave will consider the forms of risk outlined in Section 2 and include the views of the patient, carer/s, family and the multidisciplinary team.

The psychiatrist can require the consumer to reside in a particular place, receive specific treatment or attend a particular place such as the community mental health service and remain there until they receive treatment during the period of leave.

The granting of leave must be stipulate whether it is for escorted or unescorted leave.

Overnight leave for involuntary patients is to be documented in accordance with the MHA 2014 and documented on a Form 7A Grant of Leave to an Involuntary Patient.

The granting of day leave for involuntary patients is to be documented in the patients' medical record, utilising the [AKG Grant of Leave form AKMR55.6](#), identifying the period of leave granted, the frequency of the leave and any restrictions that are necessary.

All episodes of leave must be documented contemporaneously (i.e. at the time) for the safety of the individual patient and for information in the event of an internal emergency.

When considering whether to grant a leave of absence to an involuntary patient, a psychiatrist should consider whether voluntary status or a Community Treatment Order (CTO) would be more appropriate.

Length of leave may range from minutes to several days. Leave extended for longer than three days is to prompt a multidisciplinary review about whether discharge is more appropriate.

Granting leave of a period longer than overnight is an event that requires that the consumer's nominated person, carer or close family member be notified unless the Psychiatrist determines it is not in the consumer's best interests.

8. Escorted leave

If the psychiatrist authorises escorted leave, then the person(s) considered suitable to provide the escort must be specified by name or by role in the granting of leave.

The appointment of an escort should be consistent with the wishes of the patient unless the:

- Nominated escort does not consent to escorting the patient; or
- Psychiatrist is concerned that the person nominated as escort is likely, based on current risk assessment, to be placed in a situation they are unable to manage during leave.

The patient's next of kin / carer / support person or another identified responsible person can escort the patient at the treating psychiatrist's discretion, subject to them agreeing to the conditions of the leave.

The escort is to be provided information with regards to the expectations of their role. This is to include awareness of:

- The duration of leave and any conditions in place.
- The need to stay with the patient for the duration of the leave.
- The need to be able to contact the hospital, and to be contacted, by mobile phone during the leave.

- The need to debrief with nursing staff on return of the patient to the ward.
- Actions to take if during a period of leave the patients a risk to themselves or others; including that in the event of a serious imminent threat to/from the patient or the public arises, the escort is to contact the police for assistance.

If the proposed escort person does not agree to the above conditions, the grant of leave may be withdrawn until alternative arrangements can be made.

8.1 Escorted leave by clinician

It may be necessary or desirable for a clinician to conduct the escort.

A risk assessment and a clinical handover, as per [AKG Clinical Handover Policy](#) and in accordance with the [WA Health Clinical Handover Policy](#), must take place before and after all leave escorted by a clinician.

Staff must take the ward mobile with them whilst [Chaperoning](#) patients.

Escorted leave by a clinician is to be provided in the manner prescribed by local policy or procedure and is subject to:

- The patient's need for supervision.
- The capacity of the service to provide an escort, given the existing or anticipated clinical acuity.
- The availability of suitable non-staff escorts.

In the event that a patient becomes aggressive / violent during leave, the escort is to take reasonable steps to ensure the supervision and safety of the patient only after they have ensured their own personal safety. If, during a period of leave, serious imminent threat to/from the patient, or to a staff member or the public arises, staff are to contact the police for assistance.

9. Staff consultation

The patient's allocated nurse or the Shift Coordinator must be consulted before any patient is escorted from the ward by any staff member of any profession.

Clinical Discretion

Mental health clinicians are to exercise their discretion and may decline leave that has been granted, pending review by a doctor, if it is assessed that there has been a change in the risk assessment or an adverse change in the patient's mental state or physical condition.

A refusal of leave can precipitate distress, threatened or actual self-injury, aggression and absconding. If leave is not considered appropriate staff will:

- Communicate this decision to the consumer as tactfully and positively as possible at an appropriate time and in a suitable setting.
- Develop alternative strategies to address the consumer's expressed needs (eg. If spending money is required, a family member can be asked to bring some to the ward).

This situation, and the rationale for revoking approved leave, is to be documented in full in the patient's medical record and discussed with the patient and carer / family.

10. Review of patients on return from leave

Following the consumer's return from leave, staff will routinely ask the consumer how they felt the period of leave went including:

- Did the patient feel the period of leave taken was successful?
- How the patient felt whilst they were on leave? Did they feel safe?
- Would the patient be confident if they were to have another period of leave?
- If appropriate, ask the patient's PSP/family how they felt the leave went.

Document feedback in the Medical Record.

11. Documentation

The responsible mental health clinician must document any leave taken at the time of leave in the digital medical record. Staff are to also ensure the [AKG Grant of Leave form AKMR55.6](#) is completed to facilitate this.

Documentation must be contemporaneous and is to include:

- The patient's status under the MHA 2014;
- The clinical risk assessment and safety / risk mitigation plan, utilising the RAMP;
- Communication of the approved leave, the purpose, duration and form of leave, any escort details, special conditions of leave, specific instructions for continuing care and contact details of the escort and the patient;
- Accommodation and/or destination and transport arrangements during the leave period;
- Any medications or equipment given to the patient to take during their leave period;
- The time leave commences and the time of returning;
- Handover of leave arrangements to the next shift where applicable;
- Any notable incidents / events which are reported to have occurred during the leave, including any deterioration in the patient's physical health status or mental condition and any debriefing or other actions that followed return from leave;
- Where applicable, the reason for the patient not returning by the agreed time and actions thereafter.

12. Patient missing, AWOL or fails to comply with granting of leave conditions

If the patient goes missing whilst on leave, fails to comply with any conditions attached to the granting of leave or is missing from the IPU, this is to be reported immediately to the Ward Coordinator / Associate Nurse Unit Manager or Nurse Unit Manager and the responsible medical staff.

The patient's Care Coordinator is to be informed of patients that do not return from leave and are currently active with the community service.

Regardless of the patient's status under the MHA 2014, if a mental health inpatient on leave does not return at the agreed time and cannot be contacted to establish their safety, the patient will be considered as missing. In these circumstances, actions as prescribed in the [EMHS Missing or Suspected Missing Inpatient Policy](#) are to be initiated.

13. Roles and responsibilities

The following outlines the specific roles and responsibilities relevant to this policy:

Role	Responsibility
Service Director Mental Health	Ensure that managers and supervisors are aware of the requirement to implement safe practices relating to granting and management of leave for mental health inpatients.
Nurse Coordinator	Ensure the development of relevant protocols and procedures.
Nurse Unit Manager and Associate Nurse Unit Managers	- Ensure staff are trained in relevant safe Mental Health practices. - Where available and appropriate, ensure appropriate training and education is provided to staff in relation to granting and management of leave for mental health inpatients.
Staff	- Ensure patients and carers are provided the opportunity to discuss leave arrangements prior to leave. - Document any leave taken as well as the patient's overall leave experience in the patient's Digital Medical Record (DMR). - Comply with mandatory training requirements and proactively seek further training as required. - Comply with safe Mental Health practices. - Ask for assistance if required and involve relevant medical staff in the decision-making process regarding granting and management of leave for mental health inpatients.

14. Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Granting and Management of Leave for Mental Health Inpatients policy through consultation with EMHS Aboriginal Health Strategy Team to Ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD 4093).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition and Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Island colleagues and community.

15. Compliance monitoring

Compliance against this policy will be monitored by the Mental Health Safety and Quality Committee.

Compliance monitoring activities relevant to this policy include:

- Monitoring of clinical incidents related to missing or AWOL mental health patients.
- Review of critical incidents that occur whilst a patient is on a period of granted leave.

16. Legislative context

The following documents are required to give effect to this policy:

- [Mental Health Act 2014](#)
- [DOH MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#)

- [DOH MP 0122/19 Clinical Incident Management Policy](#)
- [DOH MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)
- [EMHS Missing or Suspected Missing Mental Health Patients/Consumers Policy](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist Policy for Public Mental Health Service](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)

17. Supporting information

The following documents support the implementation of this policy:

- [EMHS Transportation of Mental Health Patients Policy](#)
- [AKG Mental Health Act Notifiable Events, Notifiable Incidents and Review of Status Procedure](#)
- [AKG Safe and Supportive Observations Procedure](#)
- [AKG Consumer Safety Planning Procedure](#)
- [AKG Tobacco \(Nicotine\) – Patient management Guideline](#)

18. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.07
- Standard 2 – Action 2.03
- Standard 5 – Action 5.03
- Standard 5 – Action 5.04
- Standard 8 – Action 8.04

19. References

1. Clinicians' Practice Guide to the Mental Health Act 2014
2. SMHS Granting and Management of Leave for Mental Health Inpatients Policy

20. Glossary

The following definitions are relevant to this policy.

Term	Definition
Absent person	A mental health patient whose whereabouts should be known but cannot be ascertained after diligent efforts are made to locate and/or contact them have been made by staff however their life or health is not believed to be in danger, and they are not believed to be at risk of harming others.
Absent without leave (AWOL)	Where an involuntary patient is absent without approved leave from a hospital or other place (MHA 2014 s.97).
Escorted Ground Access (EGA)	Leave within the health service site with supervision from a staff member due to MHA status or for duty of care.

Term	Definition
Escorted Leave	A leave of absence, where an individual is appointed to consistently accompany the inpatient throughout their leave period.
Involuntary patient	A person who is under an involuntary inpatient treatment order or an involuntary community treatment order (MHA 2014 s21).
Missing person	Any voluntary psychiatric patient at high risk of harm who is missing from a mental health service, general hospital or emergency department, without the agreement of or authorisation by staff.
Personal Support Person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness - close family member, carer, nominated person, the parent or guardian of a child, any guardian or enduring guardian of an adult (MHA 2014 s.7).
Unescorted Ground Access (UGA)	Leave within the health service site without supervision from a staff member.
Voluntary patient	A person who is receiving treatment by a Mental Health service voluntarily.

21. Policy contact

Enquiries relating to this policy may be directed to:

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22. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V1.0	05/03/2025	05/03/2028	Note: changed from a guideline to a policy

23. Authorisation

This policy has been authorised and issued by:

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	05/03/2025
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